ROUTLEDGE

Professionalism, Boundaries and the Workplace

edited by

Nigel Malin

Professionalism, Boundaries and the Workplace

By discussing professionalism as a set of workplace practices where boundaries have been redefined, this book argues that changes in professionalism are in response to 'new managerialism', the enterprise culture and a drive towards credentialism.

The chapters are based upon new research studies using experience from probation, social work, the NHS, small business and church settings. The issues covered include:

- the relationship between personal and professional values
- changing professional–client relationships
- definitions of 'being professional'
- conflicts arising from different understandings of professionalism
- the construction of professional boundaries

Professionalism, Boundaries and the Workplace will be essential reading for those studying social work and health care studies, management and policy studies and post-registration care professionals.

Nigel Malin is Professor of Community Care and Divisional Research Co-ordinator at the University of Derby.

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Introduction

Nigel Malin

Professionalism, Boundaries and the Workplace looks at professionalism as a set of workplace practices where boundaries have been redefined in response to socio-economic and cultural pressures. It uses examples principally from fields of health and social welfare, with the majority of chapters presenting new research findings. Different scenarios are depicted as a response to factors embedded in contemporary culture such as commercialism, credentialism and enterprise. The changed nature of professionalism is viewed also as a response to pressures from the Left and from the user movement. Partnerships and participation are taken as appropriate goals for professionalism, including desirability of extending service-user involvement into broader occupational groupings of caring professionals. Research studies contained in separate chapters are based on probation, social work, community care, NHS, small business and church settings. They address a number of issues including: the relationship between personal and professional values, changing professional–client relationships, definitions of 'being professional', conflicts arising from different understandings of professionalism, and the construction of professional boundaries.

The sociological literature on professions has largely treated the values associated with professionalism – altruism, personal detachment, public service, etc. – as part of the rhetoric by which professionalising groups support their claims to status. But we can also look at professionalism as a set of workplace practices, in modes of interacting with clients which stress the containment of subjective feelings (positive or negative) which a doctor, social worker, teacher, etc. might entertain for a patient/client/pupil. Professional training often provides ways in which over-identification with the client is avoided or managed, and codes of ethics which purport to police the boundaries between the personal and the professional. In some professions, boundaries are particularly hard to maintain.

For example, social and community workers, healers and counsellors can be overwhelmed by the problems their clients bring to them – resulting in 'burn out'. Chapter 2 by Katie Deverell and Ursula Sharma shows how some kinds of professional work, characteristics or experiences which are shared with the client group may be a major qualification for the job (black workers taken on to work with black client groups, gay male workers employed to work with gay males, etc.). In such cases the professional worker may need to develop

strategies to deal with the ambiguity of the boundary between 'work' and 'non-work'. The very private and relatively unsupervised nature of certain kinds of professional–client relationship may make it easier for detachment to be abandoned, as, for example, where sex takes place 'in the forbidden zone' between (say) therapist and patient, teacher and student.

The nature of professional practice, as it relates to scope, competence, level of discretion and power, has been linked to organisational culture within the wider UK and global economy. A notable example of the need for boundary redefinition arises from current links between professionalism and market/enterprise culture. The discourse of enterprise challenges occupational, functional and professional segmentation, monopoly and division; instead it celebrates integration and flexibility, the deregulation of professions and monopolies of competence. The collapse of barriers between occupations and functions within organisations means that members of different occupational groups are now required to work in multi-functional teams. This has given rise to the 'organisational professional', with a shift from productive behaviour, or inputs to productive behaviour, to a different emphasis on the total behaviour, attitudes and self-understanding of the individual employees. Managerialism's role in clipping the wings of professional groups has been backed by the previous Conservative Government's obsessive insistence that serious improvement can only be brought about by management. As such, managerialism has come to dom-inate professional culture by extending the 'technical' role of professions, including downgrading, de-skilling and task fragmentation.

The book explores the constitution of the professional field of expertise, how re-delineation of boundary contours has come about and how workplace practices have become substitutes for conventionally preconceived professional functions. The issue of professionalism as a set of workplace practices is considered within the following subcategories: professionalism and shared user identification (e.g. user self-advocacy), professionalism and enterprise culture, professionalism and new managerialism, professionalism and credentialism, and professionalism and emotion management.

In the caring services sector, professionalism and volunteerism can be seen as interacting in defining types of work, spheres of competence and modes of interaction between sectors. For instance, activities falling within the broad term 'caring' encompass the contribution of professional (i.e. paid) workers employed by non-governmental organisations, as well as the contribution of volunteers attached to any number of statutory and non-governmental agencies. This issue is explored in Part I, chapter 1, in relation to community and social care within the formal sector. Caring, when subjected to analysis, is often found to be about measuring the unmeasurable. When caring occupations are subjected to managerial-style review the different aspects of their identity often go unrecognised; this places unseen stresses on professional workers and may hamper their contributing to any struggle for power as a prerequisite to attaining a credible public identity. Moves towards multi-professionalism taking place within caring services may seem unpopular among single professional disci-

plines, despite rhetoric from politicians viewing such an objective as a fulfilment and culmination of a planned process. Chapter 3 by Steve McNally considers the ascendance of user self-advocacy and the challenge this poses to professional groups.

Part II, concerned with professionalism and enterprise culture, examines the way market values and organised competition have achieved an overriding prominence in creating professional efficiency. For instance, chapter 4 by Valérie Fournier considers the constitution of the independent and self-contained field of knowledge of the profession, illustrated with the example of the medical field and the emergence of the body as a valid object of knowledge. Focusing upon the labour of division that goes into erecting and maintaining boundaries between the professions and various groups, the argument is that much of the boundary work central to the establishment of the professions may be eroded or challenged by the spreading discourse of enterprise and its celebration of 'boundarylessness' and flexibility.

This is a theme taken forward in subsequent chapters. Chapter 5 by Matthew Gorton refers to the world of small businesses and considers links between delivering high quality services and customer satisfaction, where the latter may not always be commensurate with assisting organisational long-term survival. Here professionalism between agencies is restricted by structural and financial barriers. Chapter 6 by Helen Cameron explores the challenge posed by enterprise culture by looking at the work of a profession, i.e. the clergy – described as on the margins of society – to see what workplace practices its members use to sustain their professional mode of interacting with their clients. It demonstrates that clergy reluctance to allow overt conflict in their organisational leader role may be an attempt to safeguard their professional pastoral role with church members.

Part III, concerned with professionalism and managerialism, focuses upon social work and care management. Chapter 7 by Mark Lymbery examines how community care reforms have set in train a process which redefines the nature of the social worker/care manager's relationship with the service user. The increasingly short-term nature of social work involvement, focused largely on the process of assessment, detaches social workers from this key element of their professional identity, and gives more credence to technical and procedural requirements. Chapter 8 by Tim May and Mary Buck is based upon a research project on organisational change in a social service department, involving interviews with team managers and frontline social workers. Using Weber's concept of formal and substantive rationality, it evaluates strategies adopted by social workers to construct legitimated boundaries, given the plethora of organisational change.

Part IV, on professionalism and credentialism, considers the relative merits of competences and professional skills, including the notion of 'info-normative' control that places emphasis on the total behaviour, attitudes and self-understanding of individual employees. Chapter 9 by Tina Eadie addresses the changed professional focus of probation services embracing ideas of punishment and control,

and how the interaction with clients is increasingly determined by rules and procedures. Chapter 10 by Reva Berman Brown and Sean McCartney is based upon research findings covering attitudes and values of general managers and clinicians in the NHS with respect to differing definitions of what it means to be a professional. Because of such differences in values the interaction between, for example, medical consultants and managers is clouded by different conceptions of what a 'true' professional is, and whether he or she acts in a properly professional manner. Credentialism and women GPs is considered in chapter 11 by Ruth Pinder. This chapter looks at boundary issues around part-time working and the ambiguity and anxiety around blurring boundaries between different kinds of part-time statuses.

Part V examines emotional organisation in an enterprise culture, the extent to which professional attachment is valued and whether it crosses cherished boundaries. Chapter 12 by Sharon Bolton considers skills of the nursing profession necessary in managing situations in order to create the correct emotional climate and the different characteristics of various types of organisational emotionality. Chapter 13 by Richenda Power considers the 'fat envelope patient' and the implications for professional boundaries and emotion management which arise from those patients who are frequently referred through NHS channels to osteopaths, homoeopaths, etc. The patient as active agent in the construction and maintenance of such situations is explored, as it is in the last chapter by Steve Killigrew, which evaluates the professionalism concept in relation to complementary medicine, including ways used by practitioners to redefine the role of emotions in the therapeutic process.

Part I

Professionalism, boundaries and the health/social care context

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1 Professionalism and boundaries of the formal sector

The example of social and community care

Nigel Malin

Introduction

This chapter focuses on policy directions for social and community care within the UK and assesses change and development for the caring professions. In part, the shifting, peripheral and unspecified nature of care in the community is blamed for the failure of groups to professionalise, gain control and achieve closure status. In addition the mixed economy of care has tended to emphasise the need for staff to possess appropriate values, skills and attitudes, and for this to have greater importance than forms of academic training. A drive to 'modernise' health and social care services led by central government in the late 1990s has been accompanied by the search for an underpinning knowledge-base, and also for greater professional and hierarchical accountability. The argument is that this is creating a challenge for caring professions and is leading to the development of different types of expertise within social care.

Policy origins and developments

Social and community care arose in part as an outcome of de-institutionalisation. The 1957 Royal Commission on the Law Relating to Mental Illness and Mental Deficiency had concluded that 'it is not now generally considered in the best interests of patients who are fit to live in the general community that they should be in large or remote institutions such as the present mental or mental deficiency hospitals' (Ministry of Health 1957: 17, para. 46). Yet the Commission's vision of community care was founded on provision of a range of centres offering training, occupation or social activity and residential accommodation in private, voluntary and local authority homes and hostels. The idea of community care was not advanced as a principle during the 1960s chiefly because of lack of attention from central government and poor – almost invisible – co-ordination of services at local level. Barham's (1992) book covering mental health policy states that 'the declarations of government policy in this period represented not so much a new departure as an assertive formulation of trends that were already taking place' (ibid.: 11). Social and community care were unspecified 'simply because the character of the ex-mental patient in the community remained to a large degree unspecified' (ibid.: 12).

Community care facilities at this time were envisaged as transitional stepping-stones between a brief period of hospitalisation and full re-integration into the community. A transitional period in a rehabilitation hostel or day centre, it was believed, would accomplish the mutation of former mental health patients into ordinary citizens, distinguishable from their associates only by their occasional attendances at out-patient clinics to review their prescriptions. The new-found psychiatric optimism about the major tranquillisers, coupled with protest about the indignities of mental hospital regimes, converged to suggest that the concept of community care did not require elaborate specification or commitment (Bennett and Morris 1983). In truth the community as a locus of care came to possess little value (Barham 1992: 14); it was neither seen as a therapeutic site nor as the arena for an interrogation of the moral crisis in the relations between people with mental health problems and the larger society, but just as the place to which people were to be returned after medicine had cured them.

A by-product of the continuing non-specification of the nature and purpose of social and community care was a failure to carve out a proper role for staff as carers, including methods of working, training and service objectives. Care work linked to the public services was seen as having professional orientation but as a subordinate partner to established medical and clinical professions. Health and Welfare (Ministry of Health 1963) provided a list of services to promote social care for elderly people, including provision of suitably adapted ordinary housing, warden-supervised housing, home helps, laundry services, provision of cooked meals, chiropody at home, friendly visiting and the availability of additional transport to help people to attend social events. Many of these ideas were also taken up for younger people with disabilities in the Chronically Sick and Disabled Persons Act 1970.

Social and community care policies arose later from the 'planning for priority groups' agenda in the 1970s. Such policies demonstrated a significant gap between aspiration and intent and the means of implementation. Most of the central government master-plans published during this period for user groups such as mental health, learning disabilities and the elderly took the view that community care could be planned rationally and incrementally through 'local agency collaboration' following general objectives and principles laid down by government. In retrospect this seems naive; the model for achieving change was 'simply inappropriate' (Lewis and Glennerster 1996: 20). The government had tried to adopt centralized, rational, comprehensive planning. It had produced a national budget and planning guidelines, and local districts and social service departments were intended to plan jointly to implement these guidelines. In practice, power in the National Health Service (NHS) and local authorities was so diverse and the competing bureaucratic interests so entrenched that such a model had little hope of success; setting achievable goals to create high quality or adequate social care within the community for vulnerable, dependent service users proved unrealistic.

Local service providers muddled through; in general they extended residen-

tial and day centre services but with the emphasis on providing basic physical care and occupation. This showed a failure to connect with any desired outcome: 'policy throughout the 1970s was not incisive and continued to straddle community-oriented and institutional positions' (Felce and Grant 1998: 25). It meant also that a distinction was being drawn between treatment function and the long-term supportive function of care services. Whereas the former retained a foothold in hospital provision linked to clinical skills, the latter, as located in the community, lacked a clear purpose despite eventually being seen as an alternative 'social' non-clinical form of care. Social care as an occupational identity was disadvantaged through its failure to organise as a corporate group, and through its dependence on a third party defining and mediating contracts between client and professional worker.

In the 1980s it was widely publicised that the majority of community care was provided by family members, mainly women. Community care policies were examined against the background of an increasingly ubiquitous carers' lobby. The nature of care and who provided it became issues subject to frequent, rigorous analysis. In its original form, community care policy had envisaged a significant role for public services in maintaining highly dependent people outside large institutions. By the early 1980s this was less clearly the case (Baldwin and Twigg 1991). Under the twin pressures of fiscal crisis and an ideologically driven commitment to reducing the role of the state in service provision, the original vision was replaced by a much stronger emphasis on the provision of care by 'the community' itself (Parker 1990).

It raised the question that if care in the community was being provided by unpaid non-professionals then where was the argument for professionalising other spheres of similarly provided social care in residential homes and day centres. Within the formal sector, the notion of care was perceived as having less active connotations than certain other concepts used to describe what professionals do to their clients or patients. Terms such as treatment or therapy suggest an active intervention by a professional who has undergone training and has particular skills which qualify her to use technologies designed to achieve particular outcomes (Barnes 1997: 13). In contrast to this, care is seen to be something which can be provided without the need for professional training: the job of 'care worker' which exists in residential homes, domiciliary settings or day centres is usually an unqualified post. The provision of 'care' can imply an absence of clinical intervention directed towards cure, and when not undertaken against a background of medical technology, for instance within a hospital setting, it can be seen as activity subordinate to the work undertaken by the professionals.

Since the 1980s the problem of who provides social and community care has been taken up as a gender issue. How to 'empower' carers was central to the gender-based debate. In April 1997 service users were given power to select and employ their own carers through direct payments legislation. Ungerson (1997) has described the double disadvantage of mainly women being employed as low paid carers working in relative isolation from mainstream services, where

informal and illegal contractual arrangements were unlikely to provide them with employment rights.

Emphasis upon values and philosophy rather than forms of professional training

The intention of the community care initiative was to help long-stay hospital patients unnecessarily kept in hospital to return to the community. The 1981 Care in the Community consultative document (DHSS 1981) began with the phrase: 'most people who need long-term care can and should be looked after in the community. This is what most of them want for themselves and what those responsible for their care believe to be best' (ibid.: para. 1.1). The 'legal, administrative and financial framework within which health and local authorities operate' (ibid.: para. 1.2) was presented as the obstacle to their transfer.

The policy of the Conservative government was not to fund de-institutionalisation nationwide but to set up demonstration projects where lessons could be learned, particularly with respect to managing joint health–local authority finance. The real motive of government was to cut the formidable expense of long-term hospital care and later to promote more cost-effective, cost-efficient varieties of care through separating the role of purchaser of care from that of provider. The emphasis would be upon altering the means rather than the outcomes (unless cutting the overall state financial contribution counted as an outcome). In a 1983 circular, the government established a number of pilot projects to investigate ways of moving long-stay patients out of hospital (DHSS 1983). Twenty-eight demonstration projects were centrally funded for three years (13 in the first round which began in April 1984, and 15 in the second which commenced a year later). Most of the projects represented models of care for people with mental illness or learning difficulties, and for elderly or elderly mentally infirm people. Together these pilot projects expected to provide services which would enable about 900 hospital patients to move into the community (Renshaw *et al.* 1988: 34). As the Personal Social Services Research Unit (University of Kent) evaluation indicates, by March 1987, 456 people (only) had moved out to the community, from a planned total of 896 (ibid.: 173). The main problems concerned finance (for example, 'double funding' – transitional costs required to put a new service in place before the old one can be removed), timescales (nearly all projects found that every step took far longer than had been anticipated), and professional resistance.

Professional resistance included efforts by health service staff to slow down the pace of hospital closure, coupled with the failure to demonstrate approval for care being provided by non-clinically qualified personnel. For the majority of demonstration projects this initiative created an opportunity to begin formulating objectives for care in the community and in so doing, to recognise the significance of the philosophy of care and values held by staff. The success of any project was seen to hinge on the quality of staff (Renshaw *et al.* 1988: 134). Back in 1980 the King's Fund Centre project entitled 'An Ordinary Life' de-

cided that skills for residential care 'are mainly very ordinary ones: because staff themselves live in society and do most of these activities for themselves every day, they have the skills already' (King's Fund Centre 1980: 33). This seminally influential report on planning localised provision for people with learning disabilities concluded that most tasks which staff undertake fell into one of two categories: 'teaching' and 'doing'. It identified that staff needed training in the techniques of identifying need, deciding priorities, setting programmes and teaching skills to clients, and that 'in many cases the formal qualifications of staff and programmes of training currently provided for them are quite irrelevant' (ibid.: 33).

The 1980s Care in the Community projects emphasised in-house training for staff and evaluations focused upon its function in providing support, boosting morale and in clarifying service values and objectives. The tendency was to distance training from professional agendas, to reinforce local ownership, for staff to become partners in a local project; hence across the country much of the staff training evolved in a fragmented fashion. The PSSRU studies reported: 'our research showed that in-service training was positively related to expressed job satisfaction' (PSSRU 1990: 15).

Brown's (1992) analysis of joint training (between nursing and social work) as it applies to learning disabilities demonstrated that such modes of training arose as a result of the marginalisation of learning disabilities from mainstream services and competition for ownership between the two main caring professions. For staff training in the field of learning disabilities this divide-and-rule mentality lessened their claims to professional status and competence. When in the early 1990s government increasingly questioned occupational boundaries and addressed workforce issues, including training, in ways that challenged traditional demarcation, such collaboration between nursing and social work have provided lessons of relevance to their professions as a whole. By looking for links within and among the training for caring professions, the result had been to undermine rather than to enhance any special claim for identity. In learning disabilities, much of this had its roots in the Jay Report (Jay Committee 1979), whose remit was to recommend patterns of training, but which largely sidestepped the issue of professional alliance and concentrated upon the nature of the care task in an arguably apolitical manner: 'we did, however, feel strongly that it was a central part of our remit to look at the essential nature of the tasks of caring, without being constrained by how that task is currently allocated between particular professions or agencies' (Jay Committee 1979: 35).

Ten years later, following the Griffiths Report (1988) on community care, the policy emphasis became more blatant, recommending staff training in skills fit for the task rather than focusing upon any single caring profession. The Audit Commission, in a survey of collaboration between health and social services on implementing community care, reported that staff training was weak, and that fewer than 20 per cent of authorities provided adequate training in moving service delivery 'from "caring for" to "enabling" clients' (Audit Commission 1989: 14). When 'care in the community' was introduced, the Green Paper

consultative document (DHSS 1981) stated in a section on staffing implications that both 'health and local authority workers should be fully committed to the success of projects' (ibid.: para. 3.9) and that this should be the overriding concern in appointing staff. From this period on, policy documents focused upon how to bring about progressive new services and referred to staff having to learn new skills (see, for example, Sheffield Area Health Authority/District Council 1981; Guy's Health District 1981): 'it will be vital that staff attend specially-designed re-orientation courses' (Sheffield ibid.: para. 3.2.1).

The underlying sentiment was that professional background was less relevant than capacity and orientation to undertake the task in hand. This applied both to certain non-clinical and clinical areas, and in particular to various forms of residential care. The government-led 'care in the community' project demonstrated on a national scale the need for individual care plans, service flexibility and the removal of structural, professional barriers. It also demonstrated the importance of salient local leadership and need for government to take more responsibility if practice was to change radically.

Professionalism as applied to social and community care

As a sociologist, Macdonald (1995: 133) discusses factors which affect the position and the practice of caring professions, including mediation, knowledge, indeterminacy and patriarchy. He quotes Johnson's (1972) reference to caring professions as 'mediative'; that is, they operate in conditions where 'a third party mediates in the relation between producer and consumer, defining both the needs and the manner in which the needs are met' (ibid.: 46). Their activities are defined by the state – both the needs they are to deal with and the way in which those needs are to be met; and furthermore, their funds are provided by the state (Macdonald 1995: 134). This limits their power, their dealings with other occupational groups, and weakens their position in negotiating economic rewards. Where the product of their activities is not clearly defined, their position is weakened still further.

This latter point applies in particular to the wider community-focused, enabling role prescribed to social work following the Griffiths Report in 1988. This report followed in the tradition of the previous Barclay Report (1982) and Section 12 of the Social Work (Scotland) Act 1968 in advocating a role for social work and social services beyond that of client counselling and service provision. It appeared to implement the Barclay Report's recommendation that social work and social care were marginal to the provision of care in the community. Instead the role of social work was of a broker, an enabler, an assessor of individual needs who organised the design and delivery of care packages. The provision of care *per se* would be performed by others and be relatively mechanistic and undemanding of high levels of professional skill.

The problem of defining outcomes of caring professions relates to the issue of the knowledge-base (Macdonald's analysis refers to professions possessing esoteric knowledge). In the caring professions, however, there is a considerable

body of opinion that holds that practice is actually the more important aspect (see MacKay 1990: 34). The emphasis on knowledge derives from 'trait' theory, which relied heavily on the delineation of the characteristics which were held to constitute a profession. This formed the dominant perspective in early studies of professionalism (Greenwood 1957; Carr-Saunders and Wilson 1962; Etzioni 1969; Toren 1972) and one which reflected the views of established professions. Each occupation to be considered as a candidate for the label 'professional' could be compared to the list of traits, and the degree to which it matched was then taken as an indication of the extent to which that occupation was professionalised.

Hugman (1991: 104) states that one of the principal reasons why the trait theorists identified nursing, social work and the remedial professions as 'semi professions' is that they do not appear to have developed dominance in discrete areas of knowledge. Hugman quotes Howe (1986: 96) in claiming that 'professional attributes are the symptom and not the cause of an occupation's standing'. According to this view, professionalism is thus limited by the success in gaining power over such factors as an area of knowledge and associated autonomy, rather than limited by the intrinsic nature of those factors.

Indeterminacy concerns the notion that the greater the element of judgement required in the exercise of professional knowledge, the less likely it is that the professional tasks will be open to routinisation and inspection. Both nursing and social work require considerable exercise of judgement in their practice. Macdonald (1995: 135) takes the view, however, that the exercise of judgement in these cases is on the basis of knowledge that both the lay public and adjacent professions may not see as being sufficiently esoteric to take it into the true realm of 'indeterminacy'; being 'everyday' rather than professional knowledge. This view is possibly too generalised and fails to take account of the body of theoretical work which now underpins professional practice. Regarding the specific domains of social and community care, for instance, residential, domiciliary and day care provision, the objectives are directed towards long-term promotion of client independence and rehabilitation.

The idea of patriarchy has been claimed as significant in influencing the identity of caring professions (Larkin 1983; Witz 1992; Macdonald 1995): the values of patriarchal society are built into institutions, and its practices shape and reinforce the belief among its members that they have a vested interest in maintaining such values. Aspects of nursing and social work tasks are already socially defined as appropriate for women. Those taking up these roles accept that their position within the organisation will be strengthened and secured by adhering to their predefined role. Early developments of occupational associations, e.g. nursing, were based upon a 'probationership' model as the means of acquiring occupational knowledge; consequently the emphasis (in nursing) is on dedicated caring and concern as opposed to academic training. Patriarchy is implicit within the managerialist, employer-led approach to training in social care which created a distinctive trend, challenging the historical predominance of profession-determined education, in the early 1990s.

The mixed economy of care: managerialism versus professionalism

The Griffiths Report, *Community Care: An Agenda for Action* (Griffiths 1988), was concerned with modernising the management of community care and placing 'responsibility (for care) as near to the individual and his carers as possible' (ibid.: para. 30). The report occurred within the context of a government drive to 'reform' and overhaul public sector services (for example, health and education) so as to make professionals employed there more accountable for the work they had undertaken. The method chosen was the improvement, and by necessity the increase, of overall management, as this was perceived as the essential solution.

The report stressed the mixed economy of care, arguing that it was not necessary for social services authorities to provide all services themselves, but simply to ensure that they were provided. They should act 'as the designers, organisers and purchasers of non-health care services and not primarily as direct service providers, making the maximum possible use of voluntary and private sector bodies to widen consumer choice, stimulate innovation and encourage efficiency' (ibid.: para. 1.3.4). Radical changes in the workforce were needed, involving significant changes in role for a number of professional and occupational groups. There was a further argument taken up from the earlier Audit Commission report (1986), namely the creation of a new occupation of 'community carer' to undertake 'the front-line personal and social support of dependent people' (Griffiths 1988: para. 8.4). This provided a case for developing new skills among existing staff to enable them to tackle the new tasks proposed, developing completely new roles and replacing those that no longer appeared to be necessary.

The White Paper *Caring for People* (DoH 1989) constitutes the government's response to Griffiths and this was followed by enactment of the NHS and Community Care Act 1990. Wistow (1990) described the purpose of the White Paper as operating at three levels: the macro (or service system) level; the micro (or individual user) level; and the inter-agency level. Encouraging local authorities to focus upon a needs approach was to be achieved through linking *planning* for the whole community with *planning* at individual client level. Wistow's model implies:

- the organisation of service systems based on the separation of purchaser and provider functions, the promotion of a mixed economy and the creation of new providers operating in an external market;
- the organisation of service delivery through systematic arrangements for assessment, care management, devolved budgeting and, hence, some degree of decentralised purchasing; and
- a re-emphasis on joint working through the clearer allocation of responsibilities for health and social care, combined with strengthened financial incentives for collaboration and a focus on planning outcomes rather than structures or processes.

One effect of community care policy reforms has been the de-professionalisation of the workforce: a reduction of professionally trained staff in hands-on care and in areas outside management and need assessment (see, for example, Hatfield and Mohamad 1996 on community mental health support teams). Sheppard (1995: 75) refers to the 'marginalisation of a core of tasks centred around sentimental work such as interprofessional skills and reflective responses brought about by a different style of culture dominating health and social care'. As the Griffiths Report envisaged,

> the change in role of social service authorities might also allow them to make more productive use of the management abilities and experience of all their staff, including those who are not qualified social workers . . . the professional skills of community nurses and health visitors need to be effectively harnessed and their contribution in working with other professional groups fully recognised.
>
> (Griffiths 1988: paras 8.1, 8.3)

The separation of management from professional work dealing with clients – with the former dominating professional culture – has been accompanied by a belief that such management expertise is the sole legitimate criterion for decision making in public organisations.

Griffiths (ibid.: para. 8.4) recommended: 'the creation of a new occupation of "community carers" to undertake the frontline personal and social support of dependent people . . . such job descriptions [should] enable individual workers to provide the assistance required without demarcation problems arising'. There has since been some work supporting the claim that unqualified workers demonstrate greater flair and innovation in undertaking hands-on work (Cole and Perides 1995) and that 'quality' is more associated with workplace ethos than with training and qualifications of staff (Wilding 1994). The division between managerial and professional/quasi-professional tasks has had a knock-on effect of deconstructing traditional professional competence (or professionals' right of ownership) and substituting routinisation, rule-following and form-filling. Many professionals, for example clinicians, have traditionally enjoyed high levels of autonomy, reflecting both primacy of expertise over political control and the complexity of delivering services. The impact of managerialism and the growth in power of accountancy over health professionals have demonstrated how this autonomy and hegemony is now challenged, and there is evidence of de-skilling, task fragmentation and centralisation of work planning in both health and social care.

A further jolt to the 'cognitive superiority' argument used to recognise the essence of the professional claim (see Hughes 1971: 375) has been the impact of consumerism, where professionals are 'now being asked to share their knowledge with unqualified users and to join with these users in reaching care decisions' (Wilson 1995: 8). This has been interpreted as a form of devaluing professional training. The fact that consumers should be given considerably

Figure 1.1 An example of a competence-based approach

Methods of client assessment: A1

Element A1.1 Gather and collate information from sources other than client

Performance criteria
1 His/her role is established with the appropriate persons.
2 The purpose and methods of the assessment are established with the appropriate persons.
3 Available factual information and views relevant to the assessment are obtained from appropriate sources.
4 Information is checked for accuracy where necessary.
5 Information is noted accurately, collated and stored in an appropriate form.

Element A1.2 Observe and discuss with the client

Performance criteria
1 The client's behaviour is continuously observed and recorded and valid conclusions drawn from the evidence.
2 The client's views relevant to the purpose are obtained.
3 Relevant factual information is obtained from the client.
4 All information is summarised and agreed with the client, collated and stored in an appropriate form.

Source: Care Sector Consortium (1990).

greater say in defining their own needs 'firmly places [professionals'] area of work, at least in part, in the arena of matters of everyday concern and competence. The need for any particular expertise is concomitantly reduced' (Sheppard 1995: 77).

Consequences for training

Training policy in residential, domiciliary and day care became directed during the 1990s towards a competence-based approach, a method of behaviour and performance assessment using core performance descriptors. An example is given in Figure 1.1.

It was intended that evidence be collected through observation and oral questioning to gather 'proof' of a carer's skill acquisition. Together with other factors this signalled a pendulum shift away from 'indetermination' towards 'technicality' (see Jamous and Peloille 1970 for an explanation of indetermination–technicality ratio, where for example if there is insufficient indeterminacy, the occupation is likely to be perceived as a practical vocation). Despite the original antipathy of the nursing profession towards contributing to the development of competence-based training (Brown 1994), 'competence' became the currency in which training programmes were expressed. Employer-led training which challenged the historical predominance of profession-determined education was promulgated by central government (for example,

Figure 1.2 Levels of competence in nurse and social work training

Level 1 Occupational competence in performing a range of tasks under supervision.

Level 2 Occupational competence in performing a wider range of more demanding tasks with limited supervision.

Level 3 Occupational competence required for satisfactory, responsible performance in a defined occupation or range of jobs.

Level 4 Competence to design and specify defined tasks, products or processes and to accept responsibility for the work of others.

Source: Brown (1992: 362).

by involving employers at a regional level in training and enterprise councils (TECs), and in training health care and social workers). A question might arise as to whether a coherent staff training policy for community care existed. Implementing the reforms gave purchasers and providers the opportunity to reflect on staff training policies and the workforce infrastructure, which involved redeploying care staff and considering how staff might acquire new knowledge and skills. Some local authorities undertook a training need analysis of their workforce (e.g. Bradford and North Yorkshire Social Services Departments).

One reason for the move towards de-professionalisation was that the contractual environment meant that traditional patterns of service provision and training programmes were no longer sacrosanct. The discernible trend was for local managers to develop their own specific training courses rather than patronise courses offering a regional, if not national, expertise, thus granting a feeling of local ownership. This raises the issue of comparability in standards between courses in different localities. Professional training bodies, for example English National Board for Nursing, Midwifery and Health Visiting (ENB), Central Council for Education and Training in Social Work (CCETSW), were expected to take a back seat, as new National Vocational Qualifications (NVQs) were devised following the government White Paper *Working Together: Education and Training* (DoH 1986). In community care, the emphasis was to be on 'pre-qualifying' (Levels 1 to 4) in the context of nurse and social work training (see Figure 1.2).

New NVQs were devised to meet demands of needs-led assessment and care planning, involving wide-ranging consultation with users and carers. There have been studies illustrating how 'unqualified' (non-professional) staff 'appear to be working in appropriate ways' (Hatfield and Mohamad 1996: 217) and contribute significantly to the support of people with chronic psychotic illness living in the community; personal qualities have been seen as more important than training. As stated earlier, this has contributed to a perception in some quarters that anyone can perform a care task: this has been disempowering to staff trying to promote the professional training argument (see Key 1995).

Modernising social and community care

> Government can act in partnership with agencies in civil society to foster community renewal and development. The economic basis of such partnership is what I shall call the new mixed economy . . . The state should expand the role of the public sphere, which means constitutional reform directed towards greater transparency and openness, as well as the introduction of new safeguards against corruption.
>
> (Giddens 1998: 69, 73)

The Government White Paper entitled *Modernising Social Services: Promoting Independence, Improving Protection, Raising Standards* (DoH 1998) addressed the issue of raising standards, improving quality and devising a national framework. The focus was upon regulation, and while recognising that around 80 per cent of those employed in social care have no recognised qualifications or training, it recommended setting standards of practice and conduct in the workplace. The establishment of eight Commissions for Care Standards, taking inspection from local authority control, was intended to have a broader scope for regulation than the current regimes. The overall approach of the White Paper is on 'the quality of services experienced by, and outcomes achieved for, individuals and their carers and families' (ibid.: 1.7).

The device chosen by the Labour government is to create a General Social Care Council to regulate training and to set conduct and practice standards for all social services staff. A desired effect would be a general improvement in ethical standards in the workplace. Provider agencies are likely to be advantaged by the employment of more trained personnel who are able to defend workplace practices during inspection. The approach follows that adopted within the NHS on clinical governance referring to forms of administrative or regulatory capacities. Clinical governance entails:

> arrangements [which] secure that the highest clinical standards are embedded in the organisation's wider quality assurance systems, and that research and development and support for continuing professional development are aligned with priorities for quality improvement, [involving] well-established systems to monitor and audit clinical standards and programmes to identify and reduce clinical risks.
>
> (DoH National Health Service Executive 1998: 3.18)

Care provided by a trained and skilled workforce is one of a number of key principles identified by the White Paper which are designed to underlie a high quality and effective service (Davis 1999). Other themes include: protection, co-ordination, inflexibility, clarity of role, consistency, eligibility criteria and inefficiency. The emphasis within the New Labour programme is on defining objectives at national level and setting outcomes leaving local agencies to set an enabling framework. The government's modernisation programme for health